

TB-500 — Subcutaneous Injection Instructions

NOT CURRENTLY OPERATIVE — This protocol is a future-state document. Bloom Metabolics does not currently dispense TB-500. This protocol becomes operative only after ALL of the following conditions are met: (1) LegitScript Healthcare Merchant Certification is obtained, (2) SP Bio Labs achieves 503A licensure for TB-500 OR an alternative compliant compounding pharmacy partner is engaged, AND (3) the Covering Physician approves this protocol for clinical use. Until those conditions are satisfied, this document has no governing authority and no patient shall be provided TB-500 or directed to these instructions.

PROVISIONAL CLINICAL CONTENT — When this protocol becomes operative, it will enter Provisional status pending full clinical review within 180 days per SOP-001 §5.6. The clinical parameters below are drafted using current evidence-based defaults and are subject to Covering Physician modification.

Protocol ID: Protocol-005

Medication: TB-500 (Thymosin Beta-4 Fragment)

Route: Subcutaneous (SubQ)

Version: 0.1

Effective Date: TBD (post-LegitScript certification)

Reviewed by: Pending Covering Physician Review

Regulatory Note

The peptide therapy compounding regulatory landscape is evolving. TB-500 (Thymosin Beta-4 fragment) is currently subject to FDA enforcement discretion regarding compounding eligibility. This protocol will be revised when:

- SP Bio Labs obtains 503A licensure specifically covering TB-500 compounding, OR
- An alternative FDA-registered compounding pharmacy partner capable of legally compounding TB-500 is engaged
- LegitScript Healthcare Merchant Certification is obtained for Bloom Metabolics
- The Covering Physician reviews and approves the clinical content of this protocol

The MSO Owner monitors the regulatory environment and will initiate protocol activation when all prerequisites are met.

Before You Begin

Gather the following supplies before starting:

- & TB-500 vial (reconstituted — check expiration date and reconstitution date)
- & Insulin syringe: 29-gauge to 31-gauge, 5/16" (8mm), 0.5mL or 1mL barrel
- & Alcohol swabs (2 — one for the vial top, one for the injection site)
- & Sterile gauze pad
- & FDA-cleared sharps container
- & Clean, flat surface with good lighting

Before every injection:

1. Wash your hands thoroughly with soap and water for at least 20 seconds
2. Inspect the vial — the reconstituted solution should be clear and colorless
3. Check the expiration date and reconstitution date on the vial
4. If the medication has been stored in the refrigerator, it may be injected cold or allowed to reach room temperature

If something does not look right, do not inject. Contact your care team.

Reconstitution

TB-500 is typically supplied as a lyophilized (freeze-dried) powder requiring reconstitution with bacteriostatic water:

1. Clean the tops of both vials (TB-500 powder and bacteriostatic water) with alcohol swabs
2. Using a syringe, draw the volume of bacteriostatic water specified by your compounding pharmacy
3. Inject the bacteriostatic water slowly into the TB-500 vial, directing the stream against the glass wall
4. Gently swirl until fully dissolved. Do not shake.
5. The solution should be clear and colorless. If cloudy or particulate, do not use.
6. Write the reconstitution date on the vial label
7. Refrigerate immediately after reconstitution
8. Use within the timeframe specified by your compounding pharmacy

Identifying Your Syringe

This protocol uses an insulin syringe. This is a small syringe with a fixed, very thin needle.

Feature	Insulin Syringe (USE THIS)	Intramuscular Syringe (DO NOT USE)
Barrel size	0.5mL or 1mL	1mL or 3mL
Needle	Fixed, 29g–31g, short (5/16")	Detachable, 23g–25g, longer (1 inch)

If your shipment contains intramuscular syringes, do not use them.

Injection Sites

Recommended Sites

- Abdomen
The fatty tissue of the abdomen, at least 2 inches from the navel
- Anterior Thigh
The front of the thigh, middle third between the knee and hip

The back of the upper arm, between the shoulder and elbow. May require assistance.

Site Rotation Schedule

Rotate injection sites with each administration. Alternate sides and locations.

Week	Site	Side	Date	Notes
1	Abdomen	Right	_____	_____
2	Anterior Thigh	Left	_____	_____
3	Abdomen	Left	_____	_____
4	Anterior Thigh	Right	_____	_____

Step-by-Step Injection Technique

Drawing Medication

1. Clean the vial top with an alcohol swab
2. Draw air equal to your prescribed dose volume into the syringe
3. Insert needle into vial, inject air, invert vial
4. Draw your prescribed dose
5. Check for and expel air bubbles
6. Remove needle from vial

Injecting

1. Clean injection site with alcohol swab — allow to air dry
2. Pinch a fold of skin at the injection site
3. Insert needle at 90 degrees into the pinched fold
4. Inject slowly over 5–10 seconds
5. Wait 5 seconds before withdrawing
6. Withdraw needle, release skin pinch
7. Apply gentle pressure with gauze — do not rub

After Injection

1. Dispose of syringe in sharps container immediately
2. Record in rotation tracker

What to Expect After Injection

Normal: Small drop of blood, slight redness, mild injection site firmness resolving within days.

Contact your care team: Worsening redness/swelling beyond 3 days, drainage, fever, allergic reaction symptoms.

Troubleshooting

Blood after injection. Normal — apply gentle pressure.

Bent needle. Discard and use new syringe. (FUTURE)

Missed dose. Administer when remembered. Do not double dose. Contact care team if unsure.

Site reaction beyond 3 days. Contact care team.

When to Contact Your Care Team

Contact via the OptiMantra patient portal for: injection site infection signs, allergic reaction, uncertainty about technique, any situation not covered here.

Emergency: Call 911 immediately for difficulty breathing, severe allergic reaction, or loss of consciousness.

Storage and Disposal

- Refrigerate after reconstitution at 36–46°F (2–8°C)
 - Do not freeze
 - Use within the timeframe specified by your compounding pharmacy
 - Protect from light
 - Dispose of needles/syringes in FDA-cleared sharps container
 - Return unused medication to pharmacy
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Acknowledgment

Before your first self-injection, you will acknowledge receipt and understanding of these instructions in the OptiMantra patient portal.

"I have reviewed the injection instructions for TB-500. I understand the technique, the injection sites, the rotation schedule, and the signs of complications. I will contact my Bloom Metabolics care team via the patient portal with any questions before injecting."

Clinical References

- Current peer-reviewed literature on Thymosin Beta-4 peptide therapy (clinical evidence is emerging; no FDA-approved indication at time of drafting).
- CDC Guidelines: Injection Practices — Subcutaneous Injections (current).
- Compounding pharmacy-specific guidance for TB-500 reconstitution and storage.

Note: TB-500 does not have FDA-approved prescribing information. Clinical evidence is based on preclinical studies and emerging clinical literature. The Covering Physician assumes responsibility for the clinical decision to prescribe this peptide under their professional judgment.

This protocol is governed by SOP-004 (Patient Self-Administration Injection Education).

Protocol-005 · Version 0.1 · Generated 2026-05-03

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